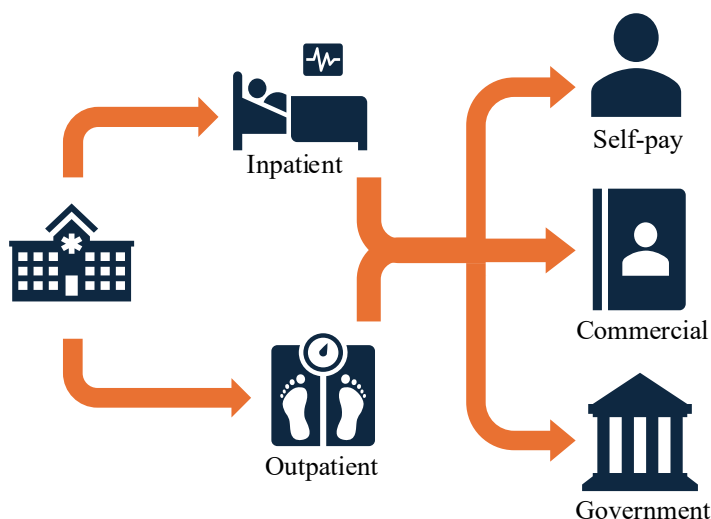


Subsector Overview

The function of this subsector is relatively straightforward. Companies in the hospital services sector provide both inpatient and outpatient care while employing physicians, nurses, and support staff to operate their facilities. These companies own the hospitals themselves, as well as the medical equipment used in operations and patient care. Hospitals generate revenue by charging for procedures such as surgeries, inpatient stays, and diagnostic services like X-rays and imaging scans. Patients are generally categorized as either inpatients or outpatients. Inpatients are patients who stay overnight in the hospital, undergo surgery, or receive intensive treatment. While outpatients receive care that does not require an overnight stay, such as same-day procedures, imaging and diagnostic services, or scheduled elective treatments. The payments that hospitals receive for their services are primarily from commercial insurers, Medicare, and Medicaid, and a smaller portion comes directly from patients through 'self-pay'. Financial performance in this subsector is driven by patient volume, the complexity of care provided, and payer mix. Typically, the more complex procedures are the higher reimbursement, while higher patient volumes help hospitals spread fixed costs more efficiently.

Payer mix is especially important, as commercial insurers generally reimburse hospitals at higher rates than government programs. When a patient is commercially insured, the payment comes from private insurance providers or employer-sponsored health plans. When a patient is covered by the government, reimbursement comes through Medicare, funded by the federal government, or Medicaid, which is jointly funded by the state and federal governments and serves lower-income populations.



HCA Healthcare: Company Overview and Subsector Alignment

Efficiency-Driven Margins



HCA Healthcare (HCA) fits into the Healthcare Services subsector as a scaled hospital operator, applying the standard hospital business model across a large network of facilities. The company owns and operates its hospitals, provides care to both inpatient and outpatient populations, employs physicians, nurses, and support staff, and invests heavily in medical equipment and infrastructure. These activities represent the core functions of the hospital services subsector. At a high level, HCA generates revenue in the same way as hospitals across the subsector. Patients receive care, and payment is made on their behalf by commercial insurers or government programs such as Medicare and Medicaid. This reimbursement flows to HCA, which then uses the revenue to pay labor, purchase supplies, and cover operating expenses. HCA captures value within this model by optimizing its margins rather than changing the underlying structure. The company increases efficiency by keeping its hospitals busy, focusing on higher-paying procedures, and using its size to control costs. Together, these factors allow HCA to generate margins that are stronger than those of many smaller or nonprofit hospital operators.

Cashflow through HCA Hospitals

The flow of cash begins when a patient enters an HCA facility and receives inpatient or outpatient care. At this stage, no cash is exchanged. Instead, the services provided are documented and coded for billing purposes. After care is delivered, HCA submits a claim based on diagnosis codes, procedure codes, length of stay, and the complexity of care provided. These claims are typically sent to one of four payors: commercial insurers, Medicare, Medicaid, or the patient directly.

Reimbursement rates are negotiated in advance with commercial insurers and set by statute for Medicare and Medicaid. Commercial payers generally reimburse at the highest rates, while Medicaid reimburses at the lowest rates. Once payment is received, accounts receivable are converted into cash, and the funds are deposited into HCA's operating accounts. During 2024 and 2025, this process generated approximately \$70-76 billion in annual patient service revenue.

Once cash is collected, it is used to fund HCA's day-to-day operations, including employee salaries, medical supplies, utilities, and contracted services. Operating cash flow reflects how efficiently HCA converts patient care into usable cash. The largest single cash outflow is labor, which includes compensation for physicians, nurses, clinical staff, and administrative personnel. In 2025, labor costs exceeded \$33 billion. Wage inflation and staffing shortages have increased overtime and reliance on contract labor has directly pressured their margins.

The next major outflow is payments to suppliers and vendors, including medical devices, pharmaceuticals, surgical supplies, IT systems, insurance, and utilities. These costs totaled approximately \$11.5 billion in 2025. Though HCA benefits from its scale, which allows it to negotiate lower unit costs compared to smaller hospital operators.

Cash is also used to meet financial obligations, including interest payments on debt and income tax payments. Together, interest and taxes totaled approximately \$4.4 billion. After operating and financial obligations are met, HCA reinvests cash into the business through capital expenditures, including new hospitals, facility expansions, equipment upgrades, and technology systems. Capital expenditures totaled approximately \$5.0-5.4 billion annually between 2024 and 2025, sustaining the physical infrastructure required to generate future revenue.

Finally, excess cash is returned to shareholders through share repurchases and dividends. In fact, cash spent on buybacks has exceeded spending on new facilities, signaling management's confidence in the maturity and stability of HCA's hospital network. When internal cash generation is insufficient or when strategically advantageous, HCA raises external capital through long-term debt issuance or the sale of non-core assets. This external capital supplements operating cash and is used to fund share repurchases, acquisitions, or capital projects.

